PRISMATIC ANALYSIS

COVID-19 Vaccination Discourse, 2020–2024

An Application of the Prismatic Method

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STEP 0: PRE-ANALYSIS REFLEXIVE DECLARATION

i. Home Grammar

My default analytical framework for this topic is what I will call the **Public Health Institutional Grammar** — a framework that treats population-level epidemiological evidence as primary, accords institutional scientific consensus significant (though not absolute) epistemic authority, and evaluates vaccination primarily through the lens of aggregate health outcomes. This is not a neutral position. It reflects my training data, which over-represents peer-reviewed scientific literature, institutional communications, and mainstream journalistic coverage relative to grassroots counter-narratives or political economy critiques. I am, in effect, pre-calibrated toward a particular signal weighting hierarchy.

ii. Hardest Grammar to Operate

The hardest grammar for me to operate faithfully will be what I will call the **Counter-Institutional Scepticism Grammar**. The difficulty is not in understanding its arguments — they are internally coherent and well-documented — but in maintaining its signal selection rules without involuntarily importing the evidentiary standards of my home grammar. The temptation will be to process signals that this grammar admits (such as VAERS reports or pharmaceutical profit data) through institutional epistemic norms (demanding controlled studies, peer review, institutional validation) rather than through the grammar's own rules, which accord different evidentiary weight to these inputs. Faithfully operating a grammar means accepting its processing architecture on its own terms, not smuggling in external validation criteria.

iii. Likely Blind Spots

From my analytical position, I am likely to under-weight the significance of institutional trust erosion as a phenomenon with its own causal force. I may also be inclined to treat the political economy dimensions of vaccination (corporate profit, regulatory capture, intellectual property regimes) as secondary or contextual rather than as potentially constitutive features of the phenomenon itself. I may also underestimate the degree to which scientific consensus is a social product, not merely an epistemic one — that is, the degree to which what counts as evidence is itself a product of institutional architectures.

iv. Temporal Orientation

I am naturally drawn to causal-historical explanation — backward-looking analysis of how events unfolded, what produced what. I will need to resist collapsing the other grammars' temporal orientations into my own. Some grammars in this domain are

primarily prescriptive (forward-looking); others are hermeneutic (meaning-making). Flattening them all into causal narratives would be a methodological failure.

SECTION I: OBSERVATION FIELD DECLARATION

The Phenomenon

The phenomenon under analysis is the public discourse surrounding COVID-19 vaccination from late 2020 through 2024, encompassing the development, emergency authorisation, mass distribution, and evolving public reception of multiple COVID-19 vaccines. The geographic scope is primarily Anglophone (United States, United Kingdom, Canada, Australia), with global reference points where relevant. The temporal frame runs from the first emergency use authorisations in December 2020 through the transition to annual boosters and declining public urgency by late 2024.

Why This Phenomenon Produces Interpretive Divergence

COVID-19 vaccination discourse is an almost ideal case study for prismatic analysis because the divergence is not primarily a disagreement about values applied to agreed facts but a deeper disagreement about which facts are salient, how evidence should be weighted, and what the phenomenon itself consists of. For some interpretive communities, the phenomenon is "a public health intervention and its outcomes." For others, it is "a case study in institutional overreach." For still others, it is "an episode in the political economy of pharmaceutical capitalism." These are not minor framings layered over a shared substrate; they constitute different analytical objects. The signals available are abundant, ambiguous in isolation, and susceptible to radically different weighting schemes. The domain is further complicated by the speed of developments (authorisation timelines measured in months rather than years), the scale of economic stakes, the intensity of political mobilisation, and the unprecedented level of public engagement with epidemiological data.

Signal Contestation Level

This is a **contested-signal domain** of the highest order. Not merely the interpretation but the evidentiary status of core signals is grammar-dependent. VAERS data, for instance, functions as a critical safety signal for one grammar and as a commonly misunderstood passive surveillance tool for another. Excess mortality data is admitted by all grammars but its causal attribution is fundamentally disputed. Even clinical trial efficacy data — ostensibly the most standardised signal — is subject to disputes about endpoint selection, follow-up duration, and the unblinding of control groups.

Major Interpretive Communities

Three broad interpretive communities dominate this discourse. First, the **institutional public health community** and those who process evidence through

its norms: regulatory agencies, mainstream epidemiology, clinical medicine, and aligned media. Second, the **counter-institutional scepticism community**: a heterogeneous coalition ranging from libertarian civil liberties advocates through vaccine-injury awareness groups to conspiratorial anti-vaccination movements, united primarily by distrust of institutional claims and different signal-selection priorities. Third, the **political economy critique community**: academic and journalistic analysts who foreground the structural incentives of pharmaceutical capitalism, regulatory capture, and the geopolitics of vaccine distribution. These three communities correspond, imperfectly but usefully, to the three grammars I will specify.

SECTION II: SIGNAL SET

Core Signals

S1: Clinical Trial Efficacy Data

Source: Pfizer-BioNTech, Moderna, AstraZeneca phase III trial publications (2020–2021). Type: Quantitative. Ambiguity: Initial headline figures (95%, 94.1%) were point estimates for symptomatic infection at a specific time point; their subsequent decline and variant-specific performance were often processed through very different frameworks than the original figures.

S2: Emergency Use Authorisation (EUA) Timelines

Source: FDA, EMA, MHRA regulatory records. Type: Procedural/institutional. Ambiguity: The same accelerated timeline is processed as evidence of institutional efficiency (expedited but rigorous) or evidence of insufficient caution (corner-cutting under political pressure).

S3: Real-World Effectiveness Data Over Time

Source: National surveillance studies (UK SIREN, Israeli Ministry of Health, CDC MMWR). Type: Quantitative longitudinal. Ambiguity: Demonstrated waning immunity and reduced effectiveness against transmission complicated initial messaging but are processed differently depending on whether the grammar's baseline expectation was sterilising immunity or severe-disease protection.

S4: Adverse Event Reporting Data (VAERS/Yellow Card)

Source: CDC VAERS, MHRA Yellow Card Scheme. Type: Passive surveillance (unverified reports). Ambiguity: This is a *contested signal* (see below). Its evidentiary status is grammar-dependent.

S5: Excess Mortality Statistics

Source: Our World in Data, national statistical offices. Type: Demographic/epidemiological. Ambiguity: Pre-vaccination excess mortality is largely attributed to COVID-19; post-vaccination excess mortality attribution is fiercely contested across grammars.

S6: Pharmaceutical Revenue and Profit Figures

Source: Pfizer, Moderna, AstraZeneca annual reports, SEC filings. Type: Financial. Ambiguity: Pfizer reported approximately $37.8 billion in Comirnaty revenue in 2022. Whether this figure is processed as a legitimate return on innovation or evidence of profiteering depends entirely on the grammar.

S7: Government Mandate and Vaccine Passport Policies

Source: National and subnational legislation, executive orders. Type: Policy/legal. Ambiguity: Implementation and subsequent rollback of mandates are processed as either pragmatic public health tools or as evidence of overreach and subsequent correction.

S8: WHO and National Public Health Messaging Changes

Source: WHO situation reports, CDC guidance revisions, UK JCVI recommendations. Type: Institutional communications. Ambiguity: Shifts in guidance (e.g., from "prevents transmission" to "reduces severe disease") are processed as normal scientific updating or as evidence of earlier overclaiming.

S9: Social Media Platform Moderation Policies

Source: Twitter/X, Facebook/Meta, YouTube content policies 2020–2024. Type: Institutional/corporate. Ambiguity: The labelling, removal, and subsequent reinstatement of vaccine-related content is processed as responsible information management or as censorship.

S10: Public Trust in Health Institutions (Polling)

Source: Gallup, Pew Research, Edelman Trust Barometer. Type: Survey/attitudinal. Ambiguity: Declining trust is processed either as a consequence of misinformation or as a rational response to institutional performance.

S11: Vaccination Rate Disparities

Source: CDC, ONS, WHO SAGE. Type: Demographic/epidemiological. Ambiguity: Disparities across race, income, and geography are processed through different causal architectures (access barriers vs. rational hesitancy vs. historical mistrust).

S12: Myocarditis Signal in Young Males

Source: CDC ACIP, Nordic registry studies (Karlstad et al. 2022). Type: Pharmacovigilance. Ambiguity: Confirmed as a real but rare adverse event; its significance relative to COVID-19 myocarditis risk is grammar-dependent.

S13: Control Group Unblinding

Source: Pfizer and Moderna trial protocol amendments. Type: Procedural. Ambiguity: The offer of vaccination to placebo group participants early in trial follow-up is processed as ethically necessary or as compromising long-term safety data.

Counter-Signals

CS1: Vaccines and Severe Disease Reduction

Across all data environments, COVID-19 vaccines demonstrated substantial and durable reduction in severe disease, hospitalisation, and death, even as effectiveness

against infection waned. This complicates any grammar that trends toward total dismissal of vaccine utility.

CS2: Historical Vaccine Safety Record

mRNA vaccine technology, while novel in deployment at scale, was built on decades of prior research. The broader historical record of vaccine safety (smallpox, polio, measles) provides a counter-signal to narratives of unprecedented risk, though it also provides counter-examples of genuine harm (SV40 contamination, Pandemrix narcolepsy).

CS3: COVAX Failures and IP Barriers

The COVAX facility's inability to deliver equitable global vaccine distribution, combined with pharmaceutical companies' resistance to TRIPS waiver proposals, complicates any grammar that treats the vaccination campaign as purely altruistic public health.

CS4: Institutional Self-Correction

Several institutions did update guidance, pause specific vaccines (AstraZeneca, J&J), and acknowledge adverse events (myocarditis). This capacity for self-correction complicates narratives of total institutional capture while simultaneously validating initial sceptical signals.

Contested Signals

Contested Signal 1: VAERS/Yellow Card Data

Institutional Grammar: VAERS is a passive surveillance system designed to detect potential safety signals for further investigation. Reports are unverified and do not establish causation. Treating raw VAERS data as evidence of vaccine harm is a category error. **Counter-Institutional Grammar:** VAERS data, despite its limitations, represents the primary publicly accessible adverse event database. Under-reporting is well-documented (Harvard Pilgrim study estimated 1–10% reporting rates). The data constitutes a safety signal that institutions have failed to adequately investigate. **Political Economy Grammar:** VAERS's design limitations are themselves an artefact of a regulatory architecture shaped by industry influence. The system was never designed to detect the kinds of signals a mass vaccination campaign might generate. Its inadequacy is structural, not incidental.

Contested Signal 2: Post-Vaccination Excess Mortality

Institutional Grammar: Post-vaccination excess mortality is primarily attributable to direct and indirect effects of COVID-19 itself (acute infection, long COVID, delayed care). Temporal association with vaccination rollout does not establish causation. **Counter-Institutional Grammar:** The persistence and demographic distribution of excess mortality after widespread vaccination is a signal

that demands investigation, particularly in younger age groups where COVID-19 mortality risk was already low. **Political Economy Grammar:** The framing of excess mortality debates reveals how different institutions control the attribution infrastructure — who defines cause of death, who funds the studies, who publishes the results.

Contested Signal 3: "Misinformation" Classification

Institutional Grammar: Misinformation is false or misleading health information that contradicts established scientific consensus, and its suppression is a legitimate public health intervention. **Counter-Institutional Grammar:** The category "misinformation" functions as an epistemic gatekeeping device that conflates genuine falsehoods with premature but legitimate questions. Several claims initially classified as misinformation (lab-leak hypothesis, waning immunity, transmission limitations) were subsequently acknowledged as plausible or confirmed. **Political Economy Grammar:** The misinformation apparatus is a collaboration between state and platform capital that reveals structural dependencies between public health institutions and information monopolies.

SECTION III: GRAMMAR SPECIFICATIONS

Grammar A: The Institutional Public Health Grammar

Source Tradition

Rooted in evidence-based medicine (EBM), Cochrane-tradition systematic review, and post-war international public health governance (WHO framework). Draws on population-level utilitarian reasoning and the precautionary principle applied asymmetrically — more cautious about inaction (not vaccinating) than about action (vaccinating) when aggregate data supports intervention.

Required Parameters

1. Primary Analytical Variable: Population-level health outcomes (morbidity, mortality, hospitalisation rates). The grammar treats aggregate epidemiological data as causally fundamental and assesses interventions by their measurable impact on these outcomes.

2. Signal Selection Rules: Admits peer-reviewed studies, regulatory agency analyses, controlled trial data, and institutional surveillance. Privileges randomised controlled trials (RCTs) and systematic reviews at the top of an evidence hierarchy. Admits observational data with caveats. Excludes anecdotal reports, unverified passive surveillance data (raw VAERS counts), and non-peer-reviewed analyses unless corroborated. Treats social media discourse as noise rather than signal.

3. Weighting Priority: RCTs > large observational studies > institutional surveillance > expert consensus > case reports. Aggregate data is weighted above individual data. Statistical significance and confidence intervals are required for signal admission.

4. Causal Chain Type: Epidemiological causation: exposure-outcome models mediated by biological mechanism. Requires demonstration of temporality, dose-response where applicable, plausibility, and consistency across populations. Tolerates some mechanistic uncertainty if population-level evidence is robust.

5. Admissible Closure: COVID-19 vaccines were a substantially net-positive public health intervention that prevented millions of deaths and hospitalisations globally, despite imperfect efficacy against transmission and identified rare adverse events. Remaining challenges are primarily communicative and logistical. The closure range extends from “unqualified success” to “successful with acknowledged limitations,” but does not extend to “net-negative intervention” or “legitimate grounds for refusal.”

Recommended Parameters

6. Excluded Signals: Raw VAERS report counts without institutional analysis. Pharmaceutical profit data (treated as economically relevant but epidemiologically

irrelevant). Individual adverse event narratives without epidemiological contextualisation. Political motivations of public health officials (treated as ad hominem). Platform moderation policies (treated as outside the health domain).

7. Characteristic Vocabulary: "The science," "safe and effective," "follow the data," "vaccine hesitancy," "misinformation," "pandemic of the unvaccinated," "risk-benefit ratio," "real-world effectiveness," "waning immunity" (accepted after 2021), "updated boosters."

8. Temporal Orientation: Primarily prescriptive (forward-looking): how should populations be vaccinated now? Secondarily causal-historical when defending past decisions.

9. Temporal Formation: This grammar's closure on COVID-19 vaccines formed rapidly in late 2020 with phase III trial results and hardened through 2021 mass rollout. Evidence since formation (waning immunity, myocarditis signal, variant escape) has been processed primarily as *revision within closure* rather than as challenges to the closure itself. The grammar absorbed these signals by shifting its message (from "prevents infection" to "prevents severe disease") rather than questioning the fundamental risk-benefit assessment.

Grammar Boundary

Grammar A would transform into Grammar C if it began to treat pharmaceutical profit structures and regulatory incentive design as constitutive of its own evidence base rather than as contextual factors. It would transform into Grammar B if it lost confidence in its own evidence hierarchy and began according anecdotal reports and passive surveillance equal weight to controlled studies.

Grammar B: The Counter-Institutional Scepticism Grammar

Source Tradition

A heterogeneous tradition drawing on libertarian political philosophy (bodily autonomy, state overreach critique), historical episodes of institutional medical failure (thalidomide, Tuskegee, opioid crisis), grassroots patient advocacy, and the epistemology of dissent. The more rigorous wing draws on Ivan Illich's medical iatrogenesis critique and the sociology of medical authority. The less rigorous wing shades into conspiratorial thinking, but the grammar itself has a coherent analytical core that should not be collapsed into its most extreme expressions.

Required Parameters

1. Primary Analytical Variable: Institutional trustworthiness and the gap between institutional claims and observed outcomes. The grammar treats the reliability of the claim-making apparatus as causally prior to the claims themselves.

2. Signal Selection Rules: Admits all publicly available data, including passive surveillance (VAERS, Yellow Card), individual adverse event reports, institutional communications shifts (treated as admissions), financial disclosures, policy reversals, and whistleblower testimony. Notably, this grammar also admits the *same* institutional data as Grammar A (clinical trials, effectiveness studies) but subjects them to different processing: institutional data is cross-examined for methodological limitations, conflicts of interest, and discrepancies between early claims and later revisions. Social media censorship events are admitted as signals about institutional boundary-policing.

3. Weighting Priority: Institutional claim-revision discrepancies > observed individual outcomes > financial incentive structures > institutional data (admitted but discounted by conflict of interest). Personal testimony and community-reported patterns are weighted higher than in Grammar A. Absence of expected data (e.g., absent long-term safety studies) is itself treated as a signal.

4. Causal Chain Type: Intentional or structural: institutions acting under identifiable incentives (profit, political pressure, liability protection) produce claims that serve those incentives. The causal architecture ranges from *structural bias* (institutions cannot see their own distortions) to *deliberate suppression* (institutions actively conceal inconvenient data). The grammar does not require conspiracy in its rigorous form — structural incentives suffice.

5. Admissible Closure: The COVID-19 vaccination campaign involved substantial institutional overclaiming, inadequate safety surveillance, suppression of legitimate dissent, and imposition of mandates that violated bodily autonomy without sufficient justification. The true risk-benefit ratio remains unknown because the institutions responsible for establishing it were compromised by conflicts of interest. The closure range extends from "legitimate but poorly managed" to "net-harmful for significant subpopulations," but its most common closure is "we cannot trust the institutions telling us it was safe."

Recommended Parameters

6. Excluded Signals: Grammar B tends to exclude or radically discount the aggregate population-level data that forms Grammar A's core — not by denying the numbers exist but by questioning the institutional apparatus that produced them. It also tends to exclude the political economy of vaccine access (Grammar C's territory) in favour of focusing on safety and autonomy.

7. Characteristic Vocabulary: "Informed consent," "bodily autonomy," "experimental," "long-term safety data," "natural immunity," "adverse events," "under-reported," "mandate," "censorship," "moved the goalposts," "Phizer" [sic, in vernacular usage], "big pharma," "do your own research."

8. Temporal Orientation: Primarily causal-historical and hermeneutic: backward-looking analysis of institutional failures and meaning-making around

personal and communal experiences of harm or coercion. Secondarily prescriptive (restore informed consent, end mandates).

9. Temporal Formation: This grammar's closure formed in two phases. An initial scepticism crystallised around the speed of EUA processes in late 2020. It hardened substantially through 2021–2022 as mandates were imposed, social media censorship intensified, and messaging shifted. Subsequent evidence (myocarditis confirmation, AstraZeneca withdrawal, mandate rollbacks) has been processed almost exclusively as *confirmation* rather than as new information requiring updated analysis.

Grammar Boundary

Grammar B would transform into Grammar A if it began to accord institutional evidence hierarchies epistemic authority rather than treating them as products of compromised institutions. It would transform into Grammar C if it shifted its primary analytical variable from institutional trustworthiness to structural political economy — that is, from asking "are they lying?" to asking "what system produces these incentives?"

Grammar C: The Political Economy Grammar

Source Tradition

Rooted in critical political economy, science and technology studies (STS), and the sociology of pharmaceuticals. Draws on Mariana Mazzucato's work on public investment and private capture, Adriana Petryna's experimentality, Kaushik Sunder Rajan's biocapital, and broader STS scholarship on the co-production of knowledge and social order (Jasanoff). This grammar is analytically proximate to Rod's own doctoral framework and the CCAM's attention to the structural conditions under which closure is produced.

Required Parameters

1. Primary Analytical Variable: The structural political economy of pharmaceutical innovation, regulation, and distribution — specifically, the relationship between public funding of research, private capture of value, regulatory architecture design, and global access inequities.

2. Signal Selection Rules: Admits financial data (corporate revenues, public R&D funding, lobbying expenditure, patent filings), regulatory design documents, international trade and IP negotiations, COVAX performance data, access and distribution equity data, and the sociology of expertise (who gets to speak, who is silenced, and what institutional structures produce those outcomes). Also admits the epidemiological data of Grammar A and the institutional trust data of Grammar B, but reprocesses both through a structural lens.

3. Weighting Priority: Structural incentive analysis > distributional equity data > financial flows > institutional communications (read as strategic rather than informational) > individual outcome data (aggregated as evidence of structural effects). Who funded the research, who captured the profits, and who was excluded from access are weighted above clinical endpoints.

4. Causal Chain Type: Structural-systemic: outcomes are produced by the architecture of incentive systems, regulatory frameworks, and capital flows rather than by individual institutional decisions or intentional misconduct. The grammar does not require bad actors; it requires analysis of how systems produce predictable distributional outcomes.

5. Admissible Closure: The COVID-19 vaccination campaign demonstrated both the extraordinary capacity of publicly funded science to produce rapid innovations and the equally extraordinary capacity of private capital to capture the value of that innovation, producing global inequities in access, distorting regulatory processes, and generating legitimate grounds for public distrust that institutional actors then pathologised as "hesitancy" or "misinformation." The vaccination programme was simultaneously a public health achievement and a political economy failure. The closure range extends from "structurally flawed success" to "systemic capture of a public good."

Recommended Parameters

6. Excluded Signals: Grammar C tends to exclude or under-weight individual-level safety concerns (Grammar B's territory) and clinical endpoint data (Grammar A's foundation). It can acknowledge that vaccines work at a biological level while arguing that this is not the most important dimension of the phenomenon. It also tends to exclude conspiratorial narratives, treating them as symptoms of structural distrust rather than as evidence.

7. Characteristic Vocabulary: "Public investment, private capture," "regulatory capture," "vaccine apartheid," "TRIPS waiver," "intellectual property," "structural inequity," "biocapital," "manufactured scarcity," "hesitancy as rational response to structural conditions."

8. Temporal Orientation: Primarily hermeneutic and lateral: what does this episode reveal about the structural conditions of pharmaceutical capitalism? Secondarily prescriptive (reform IP regimes, public pharma, open licensing). Less interested in causal reconstruction of the pandemic itself than in what the pandemic revealed.

9. Temporal Formation: This grammar's closure formed later than the other two, crystallising through 2021–2022 as COVAX failures became apparent, profit figures were published, and TRIPS waiver negotiations stalled. Evidence since formation has been processed as confirmation of structural analysis, with occasional revision (e.g.,

acknowledging that some technology transfer did eventually occur through mRNA manufacturing hubs).

Grammar Boundary

Grammar C would transform into Grammar A if it began to treat distributional and political economy questions as secondary to clinical outcomes, accepting the institutional evidence hierarchy as primary. It would transform into Grammar B if it shifted from structural analysis to intentional-agent narratives — from "the system produces these outcomes" to "they are doing this to us."

SECTION IV: SIGNAL PROCESSING

Processing Through Grammar A: The Institutional Public Health Grammar

The institutional public health grammar opens its analysis at the point of maximum evidential clarity: the phase III clinical trial results published in late 2020. Signal S1 (clinical trial efficacy data) is processed as the primary evidentiary anchor. The Pfizer-BioNTech trial reported 95% efficacy against symptomatic COVID-19; Moderna reported 94.1%. These figures, produced under the gold-standard conditions of randomised controlled trials, are accorded the highest weight in this grammar's hierarchy. They provide the foundation on which all subsequent processing rests.

Signal S2 (EUA timelines) is processed as evidence of regulatory agility within acceptable boundaries. The grammar notes that EUA is an established regulatory mechanism, not an invention of the pandemic; that the compressed timeline reflected overlapping (not eliminated) trial phases; and that the same data standards applied as in conventional approval. The counter-signal here is real and acknowledged within the grammar's own architecture: the speed was unprecedented, and the long-term safety data that conventional approval would require was unavailable at the time of authorisation. The grammar absorbs this tension by invoking the context of a pandemic emergency — the risk of *not* vaccinating was, on available evidence, substantially greater than the risk of vaccinating with incomplete long-term data. This is a legitimate but not unassailable move: it requires accepting an asymmetric application of precaution that the grammar's own principles do not automatically mandate.

Signal S3 (real-world effectiveness data over time) is where the grammar's processing becomes most interesting. The initial messaging, shaped by trial data, emphasised near-total protection. As real-world data from Israel, the UK, and elsewhere demonstrated waning immunity and reduced effectiveness against transmission (particularly with the Delta and Omicron variants), the grammar had to perform a significant internal adjustment. The closure shifted from "vaccines prevent COVID-19" to "vaccines prevent severe COVID-19." This grammar processes this shift as normal scientific updating: initial data gave a point-in-time estimate; subsequent data refined it. But *counter-signal tension* is embedded here: the initial institutional communications did not frame their claims as provisional point-in-time estimates. Statements like "you're not going to get COVID if you have these vaccinations" (US President Biden, July 2021) exceeded what the data warranted, and the grammar's own evidentiary standards would, on reflection, have flagged this as overclaiming. The grammar absorbs this by attributing the discrepancy to communication failures rather than evidentiary failures — the data was sound; the messaging was imprecise.

Signal S4 (VAERS/Yellow Card data) is processed under strict epistemic controls. The grammar admits that passive surveillance systems are essential for detecting rare safety signals but insists that raw report counts cannot be interpreted as incidence rates or causal attributions. When processed through this grammar, the myocarditis signal (S12) emerges as a legitimate finding *because* it was identified through passive surveillance, investigated through controlled studies (Karlstad et al.), confirmed as a real but rare risk, and incorporated into updated guidance. This is presented as the system working as designed. The counter-signal is the time lag between initial reports and institutional acknowledgment, which Grammar B would process as suppression. Grammar A processes it as the necessary pace of rigorous investigation.

Signal S5 (excess mortality) is processed primarily through the lens of pre-vaccination deaths attributable to COVID-19 and post-vaccination deaths avoided. The grammar points to modelling studies (Lancet, Watson et al. 2022) estimating 14–20 million deaths averted globally by vaccination in the first year alone. Post-vaccination excess mortality in some demographics is acknowledged but attributed to long COVID, delayed healthcare, and indirect pandemic effects rather than to vaccination itself — pending further evidence. This attribution logic is consistent with the grammar's causal chain requirements (temporality alone does not establish causation), but it does place significant weight on the absence of proof rather than on the absence of investigation, which is a subtly different epistemic move.

Signals S7 (mandates) and S9 (platform moderation) are processed as regrettable but defensible public health measures taken under emergency conditions. The grammar acknowledges that mandates involve trade-offs between collective protection and individual autonomy but resolves this tension by reference to precedent (school vaccination requirements, military vaccination mandates) and aggregate benefit. The subsequent rollback of mandates is processed not as an admission of error but as a response to changed epidemiological conditions. Platform moderation is processed as a necessary if imperfect response to information that could cause tangible harm (vaccine refusal leading to preventable death).

Signal S10 (declining public trust) represents an internal counterpoint — a signal admissible under this grammar's own rules that complicates its preferred closure. If the institutional grammar is correct that vaccination was substantially beneficial and its evidence was sound, the decline in public trust in health institutions is a phenomenon the grammar must explain without recourse to "the public is misinformed." Polling data showing declining trust in the CDC, WHO, and national health agencies across demographic groups, including among the well-educated, challenges the grammar's implicit assumption that correct information produces appropriate behaviour. The grammar absorbs this by invoking communication failures, politicisation, and the corrosive effects of social media — but these

explanations risk becoming unfalsifiable if every unfavourable public response is attributed to external distortion rather than to legitimate grievance.

Grammar A Closure Statement: COVID-19 vaccines, assessed by the strongest available evidence, were a substantially effective public health intervention that prevented millions of deaths and hospitalisations. They carried real but rare risks that were identified and managed through existing pharmacovigilance systems. Institutional communications were at times imprecise, and mandate policies involved genuine ethical trade-offs, but the aggregate risk-benefit calculus strongly favoured vaccination for most population groups. Remaining challenges are primarily communicative, structural (delivery, equity), and social (trust restoration).

Note: This closure is a product of Grammar A's processing architecture — its evidence hierarchy, its causal chain requirements, its signal selection rules. It is not a verdict on reality.

Processing Through Grammar B: The Counter-Institutional Scepticism Grammar

The counter-institutional scepticism grammar begins its analysis not with the trial data itself but with the conditions under which the data was produced and communicated. Signal S2 (EUA timelines) is the first signal this grammar processes with full weight. The unprecedented speed of authorisation — vaccines developed, trialled, and deployed within a year for a novel pathological category — is processed not as regulatory efficiency but as evidence that normal safety thresholds were lowered under political and economic pressure. The grammar notes that previous vaccine development timelines were measured in years to decades, and that compressed timelines necessarily compress the temporal window for detecting rare or delayed adverse events. Signal S13 (control group unblinding) amplifies this processing: by offering vaccination to placebo participants within months of initial authorisation, the long-term controlled comparison that might have provided definitive safety data was structurally eliminated.

Signal S1 (clinical trial efficacy data) is admitted into this grammar but subjected to adversarial processing. The 95% efficacy figure is cross-examined: efficacy was measured against symptomatic infection, not hospitalisation or death; the trials were powered for a specific endpoint and duration; the absolute risk reduction (approximately 0.7–0.9%) was substantially less dramatic than the relative risk reduction headlines suggested. The grammar does not necessarily claim the trials were fraudulent but argues that the institutional presentation of their results was systematically misleading — a failure that the institutional grammar's own standards of transparent evidence communication should have flagged. The counter-signal within the grammar's own architecture is that even the adversarial reading of the trial data does suggest some degree of clinical benefit. The grammar absorbs this by

arguing that benefit for high-risk populations does not justify universal mandates or the suppression of safety concerns.

Signal S4 (VAERS/Yellow Card data) receives maximum weight in this grammar — and this is where the sharpest divergence from Grammar A occurs. This grammar processes VAERS not as a crude passive surveillance system but as the canary in the coal mine: an early-warning system whose signals were systematically dismissed rather than investigated. The grammar points to the unprecedented volume of VAERS reports following COVID-19 vaccination (more reports in 2021 than for all other vaccines in the previous 30 years combined) and argues that even accounting for stimulated reporting (people filing reports because the topic was salient), the signal was extraordinary and warranted extraordinary investigation. The counter-signal acknowledged within the grammar is that VAERS volume is indeed influenced by public attention and that raw counts are not incidence rates. The grammar responds that under-reporting is at least as significant a confounder as over-reporting, and that the institutional refusal to fund large-scale active surveillance studies constitutes evidence of a system that does not want to find what it might find.

Signal S8 (messaging changes) is processed as a cumulative indictment. The grammar constructs a chronological sequence: initial claims of high efficacy against infection → acknowledgment of waning immunity → pivot to "prevents severe disease" → acknowledgment that transmission prevention was limited → annual booster recommendations. Each shift is processed not as scientific updating but as goalpost movement — a retrospective redefinition of success to match diminishing results. The grammar's characteristic phrase — "moved the goalposts" — captures this processing. The internal counterpoint is that some revision of initial claims is epistemically legitimate, especially for a novel pathogen with emerging variants. The grammar responds that the *manner* of revision — without acknowledgment of error, without accountability for earlier overclaiming, and often accompanied by accusations of "misinformation" against those who raised the very points later conceded — constitutes a pattern of institutional dishonesty rather than honest updating.

Signal S7 (mandates) is processed as the grammar's strongest case. Vaccine mandates — employment-conditional, travel-conditional, education-conditional — are treated as a fundamental violation of bodily autonomy and informed consent, principles that the grammar traces to the Nuremberg Code and the Helsinki Declaration. The grammar processes the subsequent rollback of most mandates not as a response to changed conditions but as an implicit admission that the mandates were unjustified. Signal S9 (platform moderation) is processed in parallel: the censorship of vaccine-questioning content on major platforms, followed by the gradual relaxation of these policies (including the reinstatement of previously banned accounts after Elon Musk's acquisition of Twitter), is processed as evidence

of a coordinated institutional suppression apparatus whose dissolution confirms the legitimacy of the suppressed speech.

Signal S12 (myocarditis in young males) is the grammar's most powerful specific evidence. The confirmation by regulatory agencies of a causal link between mRNA vaccination and myocarditis, particularly in young males, is processed as vindication: this was a safety signal that sceptics identified from VAERS data before institutions acknowledged it. The grammar extends the logic: if this signal was real and initially dismissed, what other signals might be real and still dismissed? The counter-signal is that the confirmed myocarditis risk was rare and the overwhelming majority of cases were mild and self-resolving. The grammar responds that "rare" is cold comfort to those affected, that "mild myocarditis" is an oxymoron (inflammation of the heart muscle is inherently serious), and that mandatory vaccination of populations at low risk from COVID-19 (young males) with a product that carried a non-trivial risk of cardiac inflammation represents a failure of the risk-benefit calculus that Grammar A claims to prioritise.

Signal S6 (pharmaceutical profits) and S10 (declining trust) are processed as mutually reinforcing. Pfizer's $37.8 billion in Comirnaty revenue in 2022 is placed alongside the fact that public funding (BARDA, NIH, Warp Speed) substantially subsidised development risk. The grammar asks: if the public bore the development risk and the company captured the profit, and if the company has legal immunity from liability (under PREP Act provisions), what structural incentive exists for honest post-market safety evaluation? Declining trust is processed not as a result of misinformation but as a rational response to a system structured to prioritise revenue over accountability.

Grammar B Closure Statement: The COVID-19 vaccination campaign was characterised by unprecedented speed, inadequate long-term safety evaluation, institutional overclaiming followed by goalpost movement, suppression of legitimate dissent, imposition of mandates that violated fundamental principles of informed consent, and a pharmacovigilance architecture that was structurally incapable of detecting the harms it was supposed to monitor. The true risk-benefit ratio remains unknown because the institutions charged with establishing it were compromised by financial incentives and political pressure. Individuals who questioned the official narrative were vindicated on multiple specific points (waning immunity, transmission limits, myocarditis risk) and deserve acknowledgment, not continued pathologisation as "hesitant."

Note: This closure is a product of Grammar B's processing architecture — its prioritisation of institutional trustworthiness, its adversarial processing of institutional data, its weighting of individual testimony and institutional discrepancy. It is not a verdict on reality.

Processing Through Grammar C: The Political Economy Grammar

The political economy grammar begins at a different altitude from both Grammar A and Grammar B. Its opening signal is not clinical trial data (Grammar A) or institutional trustworthiness (Grammar B) but the structural conditions of pharmaceutical innovation itself. Signal S6 (pharmaceutical revenue and profits) is processed not as a peripheral fact but as a constitutive feature of the phenomenon. The grammar asks: under what structural conditions was this vaccine produced, and who benefited from those conditions?

The answer begins before the pandemic. mRNA vaccine technology was developed over decades with substantial public funding — NIH-funded basic research, DARPA funding for platform development, and the specific subsidies of Operation Warp Speed (approximately $18 billion in US government contracts and advance purchase agreements). Moderna's mRNA-1273 was co-developed with NIAID. The grammar processes these facts through its primary analytical variable: public investment produced the underlying science; private capital captured the commercial value. Pfizer's assertion that it did not accept Warp Speed development funding (though it did accept a $1.95 billion advance purchase agreement) is processed as a rhetorical strategy to maintain the narrative of independent innovation, not as a meaningful distinction. The counter-signal within the grammar is that private-sector manufacturing capacity, distribution logistics, and investment in scale-up were genuine and necessary contributions. The grammar acknowledges this but argues that the terms of the public-private arrangement — public risk, private reward, liability immunity — were not the only possible terms and reflected the structural dominance of pharmaceutical industry interests in regulatory design.

Signal CS3 (COVAX failures and IP barriers) is the grammar's central case. The COVAX facility, designed to ensure equitable global vaccine access, delivered a fraction of its promised doses in its critical first year. Meanwhile, wealthy nations stockpiled vaccines far beyond their population needs (Canada procured approximately 10 doses per capita in advance orders). The grammar processes the TRIPS waiver debate as dispositive: when India and South Africa proposed a temporary suspension of vaccine-related intellectual property protections in October 2020, they were blocked for over two years by the US, EU, UK, and Switzerland — the home jurisdictions of the major pharmaceutical companies. The eventual compromise in June 2022 was described by Médecins Sans Frontières and other access advocates as functionally useless.

This grammar processes the COVAX failure not as logistical misfortune but as a predictable outcome of a system designed to prioritise patent rights over access. Signal S11 (vaccination rate disparities) extends this analysis domestically: disparities in vaccination rates across racial, socioeconomic, and geographic lines are processed as products of structural access barriers (clinic locations, work schedules, insurance requirements, language barriers) rather than individual hesitancy.

Grammar C re-describes Grammar B's concept of "vaccine hesitancy" as an individualising frame that converts structural problems into personal failures. The internal counter-signal is that access barriers do not fully explain vaccination disparities — attitudinal factors, including the historically grounded distrust of medical institutions among Black Americans (Tuskegee, Henrietta Lacks), represent a form of rational scepticism that this grammar can accommodate structurally.

Signals S2 (EUA timelines) and S7 (mandates) are processed differently from both other grammars. The EUA process is read as a regulatory architecture shaped by decades of pharmaceutical industry lobbying: the FDA's funding structure (user fees from the companies it regulates), its revolving-door employment patterns, and the EUA framework itself as a mechanism that creates market access before conventional safety timelines are met. Mandates are processed not primarily as violations of bodily autonomy (Grammar B) but as instruments that socialise the risks of vaccination while privatising its benefits. When governments mandate a product and simultaneously grant its manufacturer liability immunity, the grammar argues, the normal market mechanism that would incentivise safety (legal exposure to harm claims) is structurally disabled.

Signal S9 (platform moderation) is processed through the grammar's structural lens as evidence of the growing entanglement of public health governance and platform capitalism. The grammar notes that social media moderation policies on vaccine content were developed in direct consultation with public health agencies and reflects a form of public-private governance that is neither purely governmental censorship nor purely private editorial discretion. This hybrid governance structure — what some scholars call "governmentality" in the Foucauldian sense — is analytically more interesting to Grammar C than the binary censorship/freedom framing of Grammar B.

Signal S8 (messaging changes) is processed by Grammar C not as goalpost movement (Grammar B) or as scientific updating (Grammar A) but as a case study in the production of public knowledge under conditions of uncertainty and commercial pressure. The grammar asks: why did initial messaging overstate efficacy against transmission? Because the regulatory and commercial incentive structure rewarded the broadest possible claims. Why did messaging shift? Because real-world evidence made initial claims untenable. Why was the shift managed as quietly as possible rather than through transparent acknowledgment? Because institutional credibility — a form of capital — is protected by minimising visible error.

Signal S10 (declining public trust) is processed as the grammar's most important structural indicator. Declining trust in health institutions is treated not as a problem of communication or misinformation but as a *rational collective response* to an institutional architecture that has repeatedly prioritised commercial interests over public accountability. The grammar argues that pathologising this trust decline as "hesitancy" or "misinformation" is itself a political act: it locates the problem in the public rather than in the institutions, foreclosing structural reform. The internal

counterpoint is that some portion of declining trust is driven by genuinely false information and conspiratorial thinking that this grammar's structural analysis cannot easily accommodate. Grammar C responds that conspiratorial thinking flourishes *because* institutional transparency is structurally inadequate — conspiracy theories fill the epistemic void that institutional accountability would otherwise occupy.

Grammar C Closure Statement: The COVID-19 vaccination campaign was simultaneously a triumph of publicly funded science and a demonstration of how pharmaceutical capitalism captures public investment for private profit, structures regulatory systems to prioritise market access over long-term safety evaluation, and creates global distribution inequities that cost millions of lives in lower-income countries. Domestic vaccine hesitancy and international access failure are not separate problems but symptoms of the same structural architecture. Institutional trust decline is a rational response to this structure, not a pathology to be managed through better communications. Meaningful reform requires structural change: public pharmaceutical manufacturing capacity, reformed IP regimes, de-linked research funding, and genuine regulatory independence.

Note: This closure is a product of Grammar C's processing architecture — its structural-systemic causal chain, its foregrounding of political economy, its weighting of distributional equity. It is not a verdict on reality.

SECTION V: CLOSURE COMPARISON

i. Closure Content

Grammar A concludes that vaccines were a net-positive intervention with acknowledged limitations. Grammar B concludes that the true risk-benefit ratio is unknown due to compromised institutions. Grammar C concludes that the vaccination campaign reveals the structural pathologies of pharmaceutical capitalism. Note that these closures are not merely different emphases on a shared assessment; they constitute genuinely different propositions about what happened and what it means.

ii. Signal Divergence

The signals producing the greatest divergence are S4 (VAERS data), S6 (pharmaceutical profits), and S8 (messaging changes). VAERS data is noise for Grammar A, a primary safety signal for Grammar B, and evidence of regulatory design failure for Grammar C. Pharmaceutical profits are irrelevant to Grammar A's clinical assessment, supporting evidence for Grammar B's conflict-of-interest analysis, and the primary analytical object for Grammar C. Messaging changes are normal updating for Grammar A, institutional dishonesty for Grammar B, and structurally predictable overclaiming for Grammar C.

iii. Exclusion Patterns

Grammar A excludes individual adverse event narratives, financial incentive analysis, and platform moderation dynamics from its core processing. Grammar B excludes distributional equity data and structural political economy from its primary analysis, focusing instead on safety and autonomy. Grammar C excludes individual-level clinical outcomes and intentional-agent narratives, operating at a systemic altitude that can miss both the tangible benefits of vaccination and the tangible harms experienced by specific individuals.

iv. Causal Architecture

The causal architectures are formally distinct. Grammar A constructs epidemiological causation (exposure-outcome with mechanistic mediation). Grammar B constructs intentional or structural-agent causation (institutions acting under incentives produce predictable claims). Grammar C constructs systemic causation (incentive architectures produce distributional outcomes regardless of individual intent). These are not different levels of the same causal story; they are different *types* of causal reasoning that cannot be straightforwardly reconciled.

v. Closure Range

Grammar A is the most prescriptive: vaccinate, boost, maintain institutional trust. Grammar B is prescriptive in a different register: restore informed consent, end mandates, investigate safety signals. Grammar C is the most diagnostically oriented: the prescription (structural reform) follows from a systemic diagnosis rather than from an individual risk-benefit calculation. Grammar A's closure range is the narrowest (net-positive with caveats). Grammar B's is the widest (from "legitimate but poorly managed" to "net-harmful"). Grammar C's occupies a distinctive middle ground: it can acknowledge clinical benefit while arguing that the system producing the benefit is structurally unjust.

vi. Closure Timing

Grammar A reached closure earliest and with the most confidence, anchored by phase III trial data in late 2020. Grammar B's closure hardened through 2021–2022 as mandates and censorship intensified. Grammar C reached closure latest, requiring financial data and distributional equity evidence that became available through 2021–2023. All three grammars have shown a tendency toward confirmation processing since their respective closures formed, though Grammar C has shown the most capacity for internal revision (acknowledging some technology transfer, some reform).

vii. Grammar Translation

Grammar A re-describes Grammar B's closure as: "A failure of risk communication that allowed vaccine-hesitant narratives to crystallise into an identity-based refusal movement, amplified by social media algorithms and political opportunism, leading to preventable morbidity and mortality among the unvaccinated."

Grammar A re-describes Grammar C's closure as: "A legitimate concern about global equity that is analytically separate from the clinical question of vaccine efficacy and should not be conflated with it. Structural reform of pharmaceutical markets is desirable but does not alter the risk-benefit calculus for individuals."

Grammar B re-describes Grammar A's closure as: "The expected output of a captured regulatory and scientific establishment that was never going to find against its own products, delivered in the vocabulary of objectivity to mask institutional self-interest."

Grammar B re-describes Grammar C's closure as: "Partially correct about the structural incentives but naively trusting of the same institutions at the clinical level; if the system is structurally compromised, why accept its clinical evidence at face value?"

Grammar C re-describes Grammar A's closure as: "Technically defensible within its own evidentiary frame but structurally naive: it treats the institutional

apparatus that produces its evidence as exogenous to the phenomenon, when it is in fact endogenous to it."

Grammar C re-describes Grammar B's closure as: "Structurally perceptive about institutional distortions but analytically underdeveloped: it identifies symptoms (overclaiming, censorship, mandates) without diagnosing the systemic architecture that produces them, and therefore tends toward personalised grievance rather than structural critique."

Ontological Check

The three grammars do not constitute the same analytical object. Grammar A is analysing *a public health intervention and its clinical outcomes*. Grammar B is analysing *an institutional apparatus and its trustworthiness*. Grammar C is analysing *a political economy of pharmaceutical innovation and its distributional consequences*. These are three different phenomena that share the name "COVID-19 vaccination." They overlap empirically but diverge ontologically. This is why cross-grammar communication is so difficult: participants in vaccine debates are frequently not disagreeing about the same thing. They are disagreeing about what the thing *is*.

SECTION VI: COUNTER-SIGNALS, LEAKAGE, AND STRESS TEST

Consolidated Counter-Signal Analysis

The counter-signals identified during processing reveal that none of the three grammars achieves closure without absorbing tensions that its architecture can contain but not resolve. Grammar A's most persistent tension is between its commitment to evidence-based assessment and its difficulty accounting for declining public trust without pathologising the public. Grammar B's persistent tension is between its adversarial processing of institutional data and the difficulty of explaining the observable large-scale reduction in severe disease and death without admitting substantial vaccine efficacy. Grammar C's persistent tension is between its systemic altitude and its difficulty engaging with the individual-level harms (clinical outcomes, adverse events) that motivate both Grammar A's and Grammar B's analyses.

Leakage Instances

All three grammars borrow from their neighbours. Grammar A borrows from Grammar C when it acknowledges "access barriers" as explanations for vaccination rate disparities — this is political economy language imported into an epidemiological frame. Grammar B borrows from Grammar A when it cites specific clinical data (myocarditis rates, trial endpoint analysis) to support its institutional critique — this is evidence-hierarchy processing imported into an adversarial-trust frame. Grammar B also borrows from Grammar C when it invokes pharmaceutical profits as evidence of institutional corruption. Grammar C borrows from Grammar B's distrust of institutional claims but reprocesses it through a structural rather than intentional-agent lens. These leakages are not defects; they indicate the points at which each grammar recognises — implicitly — that its own processing architecture is insufficient to the full complexity of the phenomenon.

Grammar Stress Test

Grammar A: Most Threatening Counter-Signal

The single most threatening counter-signal for Grammar A is **the combination of S13 (control group unblinding) and the absence of long-term active safety surveillance**. Grammar A's closure depends on the proposition that the risk-benefit ratio was known and favourable. But the unblinding of control groups within months of initial authorisation structurally limited the long-term controlled safety data available, and the absence of large-scale active surveillance (as distinct from passive VAERS reporting) means that the grammar's claim to a favourable risk-benefit assessment rests on an evidence base that is less comprehensive than its own evidentiary standards would normally require. Can the grammar absorb this?

Only conditionally: by arguing that the accumulated observational evidence from billions of administered doses provides sufficient safety data, even in the absence of controlled comparison. This is defensible but represents a departure from the grammar's stated preference for RCTs over observational data. The closure is therefore *conditional on accepting a lower evidentiary standard for long-term safety than the grammar applies to efficacy*.

Grammar B: Most Threatening Counter-Signal

The single most threatening counter-signal for Grammar B is **CS1: the durable and substantial reduction in severe disease and death, observed consistently across populations, geographies, and study designs**. If the institutional apparatus is as compromised as Grammar B argues, this finding — reproduced independently across dozens of countries, healthcare systems, and research groups — is extraordinarily difficult to explain as an artefact of institutional capture. The grammar can absorb it partially by arguing that benefit for high-risk groups does not justify universal mandates and that severity reduction does not vindicate initial overclaiming. But it cannot absorb it entirely: the observable reduction in deaths post-vaccination, replicated globally, challenges the grammar's tendency toward wholesale institutional dismissal. The closure is therefore *conditional on distinguishing between institutional overclaiming and institutional dishonesty about basic efficacy* — a distinction the grammar's less rigorous expressions often elide.

Grammar C: Most Threatening Counter-Signal

The single most threatening counter-signal for Grammar C is **CS4: institutional self-correction, particularly the pause and withdrawal of AstraZeneca and J&J vaccines in response to identified safety signals**. Grammar C's structural analysis predicts that regulatory systems captured by industry interests will systematically under-detect and under-respond to safety signals. The actual regulatory response — pausing distribution of specific vaccines based on pharmacovigilance data, ultimately withdrawing AstraZeneca from the market — represents institutional action that the grammar's structural model does not predict. Can the grammar absorb it? Partially: by arguing that these corrections occurred only under extraordinary public pressure and media scrutiny, and that less visible safety signals may not receive comparable attention. But the existence of the corrections, taken at face value, suggests that the regulatory architecture is not as fully captured as the grammar's strongest form implies. The closure is therefore *conditional on arguing that self-correction was exceptional rather than systemic*.

SECTION VII: POST-ANALYSIS REFLEXIVE NOTE

My Step 0 prediction that Grammar A would be my home grammar was largely correct. Throughout the analysis, I felt the gravitational pull toward institutional evidence hierarchies: the impulse to treat peer-reviewed data as "stronger" than passive surveillance data, to read institutional messaging shifts as "updating" rather than "goalpost movement," and to frame aggregate outcomes as more epistemically significant than individual experiences. I caught this impulse operating most forcefully when processing Grammar B's treatment of VAERS data — my instinct was to add caveats about passive surveillance limitations even while operating within a grammar that weights those data differently.

My prediction that Grammar B would be the hardest to operate was also correct, but for a subtler reason than I anticipated. The difficulty was not in articulating Grammar B's arguments — they are internally coherent and well-evidenced within their own framework. The difficulty was in maintaining the grammar's *emotional register* faithfully. Grammar B is not merely an analytical framework; it is also a grammar of grievance, of lived experience of institutional dismissal. Operating it purely as an analytical exercise risks sanitising it into something more palatable to my home grammar. I attempted to preserve its adversarial energy without caricaturing it, but I am uncertain I succeeded fully.

Grammar C proved easier to operate than I expected, likely because its structural-systemic causal architecture is closest to academic analytical conventions in my training data. However, I discovered a blind spot I did not predict: Grammar C's altitude — its systemic, structural vantage point — makes it surprisingly difficult to engage with the *human* dimensions of the phenomenon. Both Grammar A (aggregate health outcomes) and Grammar B (individual harm and autonomy) maintain contact with embodied human experience, even if in different registers. Grammar C's systemic lens can render individuals invisible behind structures, which is both its analytical power and its ethical limitation.

If this analysis were conducted from Grammar B as the home grammar, the signal set would likely look different: more emphasis on individual adverse event cases, whistleblower testimony, and the chronology of institutional claim-revision. The Grammar A essay would read as a more pointed critique of institutional credulity. If conducted from Grammar C, the signal set would foreground TRIPS negotiations, global distribution data, and lobbying expenditure, and both Grammars A and B might appear more as surface expressions of deeper structural dynamics.

What does this exercise reveal? That interpretive architectures do not merely *filter* evidence — they *constitute* it. Each grammar's closure followed logically from its parameters. None was arbitrary, and none was inevitable. The method makes visible what ordinary debate conceals: that participants in vaccine discourse are typically not interpreting the same evidence differently but processing different evidence into different objects through different architectures. The disagreement is architectural,

not merely interpretive, and no amount of "better communication" will bridge it without first making the architectures themselves visible.

SECTION VIII: METHODOLOGICAL OBSERVATIONS

The prismatic method performed well in several respects. The requirement to specify grammars with explicit parameters *before* processing signals enforced analytical discipline that a less structured approach would have lacked. The five required parameters (primary analytical variable, signal selection rules, weighting priority, causal chain type, admissible closure) proved genuinely generative: specifying them revealed structural features of each grammar that would have remained implicit in a looser analysis. The grammar boundary specification was particularly valuable — articulating what would transform one grammar into another clarified the analytical architecture of each.

The integration of counter-signals during processing rather than in a deferred section was methodologically crucial. In earlier drafts of my mental outline, I found myself naturally deferring counter-signals — producing a clean grammar essay and then appending objections. The method's insistence on integration forced a more honest representation of each grammar's internal tensions. It also made visible that every grammar's closure is conditional: not a logical entailment of its parameters but a *chosen resolution* of tensions that could be resolved differently within the same architecture.

The principal difficulty I encountered was calibrating Grammar B without caricature. The method's instruction to specify grammars that are "analytically rigorous rather than caricatures" is essential but difficult in practice for a grammar that encompasses both rigorous institutional critique and conspiratorial extremes. I resolved this by specifying the grammar at its most coherent and noting its boundary conditions, but a different analyst might have specified it differently — either more charitably or more critically. The method would benefit from explicit guidance on how to handle grammars with high internal heterogeneity: should one specify the grammar at its analytical best, at its modal expression, or at its most influential?

The contested signal category proved indispensable for this domain. Without it, VAERS data, excess mortality, and the "misinformation" category would have been processed through a single grammar's rules. Documenting what each signal *is* according to each grammar made visible that the deepest disagreements in this domain are not about interpretation but about ontology — about what counts as evidence in the first place.

One refinement I would suggest: the method might benefit from a *grammar interaction* step between signal processing and closure comparison. In practice, grammars do not operate in isolation; they define themselves partly *against* each other. Grammar B's adversarial processing of institutional data is partly a response to Grammar A's epistemic authority; Grammar C's structural critique is partly a response to both. A step that explicitly maps these reactive relationships would enrich the analysis. Additionally, the temporal formation parameter could be expanded into a fuller genealogical account of each grammar's development — not

just when it reached closure but how its closure was shaped by the closures of competing grammars.

— End of Analysis —